

Patients who arrive prepared and *recover as expected.*

Your protocol in your surgeons' own words, delivered when each instruction actually matters. Alongside the pre-op packet and the discharge folder, an orbit adds a consumer-grade digital mentor: engaging video, email, and text outreach that keeps patients prepared, reassured, and recovering as expected. No app, no login, no new FTEs.



The understanding gap: the weeks either side of the operation

None of this is decided in the OR. Each one turns on whether a patient completed the prep, followed the discharge instructions, and knew which symptom warranted a call instead of an ED visit.

2-4	Weeks	30 days	-41%
VISITS AROUND THE CASE	OF PREPARATION AT HOME	THE READMISSION WINDOW	READMISSIONS IN THE TRIAL

How an orbit provides value

- 01 **Fewer avoidable readmissions.** The costliest post-discharge failure, and the one the trial reduced by 41%.
- 02 **Fewer cancellations and delays.** A case cancelled for a missed medication hold is unrecoverable OR time.
- 03 **Fewer avoidable ED visits.** The most expensive way to answer a question the patient could have had answered at home.
- 04 **Fewer inbound calls per episode.** Every repeat call is nursing time spent re-explaining what was already covered.
- 05 **Better pre-operative compliance.** Fasting, medication holds, and skin preparation are the most common reasons a case does not go ahead.
- 06 **Higher PROM completion.** Bundled and value-based programs pay on documented outcomes. Missing scores are unbilled work.

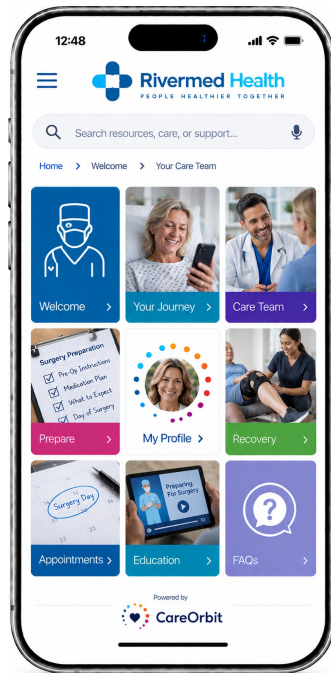
PLATFORM EVIDENCE, FROM THE CONTROLLED TRIAL

Patients arrived better prepared, called less, and came back less

-41%	-53%	+65%	+22%	9 in 10
readmissions	calls to the office	understanding of the plan and what to expect	patient satisfaction	navigated their journey better

CareOrbit controlled clinical trial, Siteman Cancer Center.

Pick one journey. *Prove it in a quarter.*



The orbit home screen as your patients see it, in your program's brand and voice. Opens in the phone's browser, with no app download required.

STEP ONE: PICK YOUR JOURNEY

Most service lines start with their highest-volume or highest-margin procedure.

- **Named procedure orbits.** Your highest-volume operations, each with its own guide.
- **Prehab and optimization.** The weeks before surgery, where cancellations are prevented.
- **Discharge and recovery.** The window where readmissions and calls are decided.
- **Procedure template.** Everything else, versioned with minor edits.

STEP TWO: SHAPE IT WITH ORBIT SOLUTIONS

Surgical orbit blueprints span nine specialties, from oncology and orthopedics to ENT and urology. Named orbits cover your highest-volume operations; a procedure template versions everything else with minor edits. We build it for your team's review.

STEP THREE: WE MEASURE AGAINST YOUR BASELINE

Live in about six weeks. We deliver engagement, completion, and the operational numbers you named, against your baseline, inside a quarter.

ZERO DISRUPTION

Nothing added to anyone's day

- **No new FTEs.** Nothing added to anyone's day.
- **Live in a quarter.** From kickoff to first patient.
- **No risk, no lock-in.** And no disruption to your offices.
- **White-label.** Your brand, your voice.
- **HIPAA, SOC 2, HITRUST.** Documentation ready for review.
- **EHR-friendly.** A scheduled flat file, or nothing at all.

FOR THE PROGRAM

You can finally see it

- Who opened their orbit and who never did, by patient
- Symptom check-ins routed back before the post-op visit
- Cancellation, readmission, and PROM completion by procedure and by site

Engagement
Index Score
What is this?

91

Every patient carries an Engagement Index Score your team can act on.

The next move: 20 minutes discussing *your* surgical pain points and *our* platform.

No demo script, no obligation. We hear which moments are costing your service line the most, and give you a brief look at the platform.

BOOK A 20-MINUTE INTRO CALL

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